

Patient ID: WheelerPeriod: May 1 to June 1, 2026Report Date: June 2, 2026Coverage: 743/745h (100%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Last 24 Hours

Elevated Breathing 2 AM to 7 AM			
Vital Stat	Avg	Min	Max
Heart Rate	59 bpm	51 bpm	71 bpm
Breathing	22 brpm	12 brpm	51 brpm

Last 24 hours: one elevated breathing episode overnight, otherwise within normal range.

Episodic Events (last 24h)

Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
2 AM to 7 AM	5h	1	Elevated Breathing	26	14	51	Check SpO2 and lung sounds; assess for fluid overload or early infection

Patient clinical status over 32 days from May 01 to Jun 01

■ HR ■ Breathing

Elevated Breathing May 01 to May 31 (31d)	High Breathing May 01 to May 01 (1d)	High Breathing May 05 to May 05 (1d)	High Breathing May 16 to May 16 (1d)	High Breathing May 29 to May 29 (1d)
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77 episodic events Detected, spanning 11 days total. Conditions: Elevated Breathing, High Breathing. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
77	278h	Elevated Breathing	2	100%

Major Findings: Sustained elevated breathing (avg 26, peak 51)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 01	7 AM to 11 AM	4h	1	High Breathing	31	17	34	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	9 PM to 10 PM	1h	1	High Breathing	32	12	49	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 16	8 AM to 9 AM	4h	2	High Breathing	31	17	36	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 29	6 AM to 7 AM	1h	1	High Breathing	32	27	36	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 01	3 AM to 7 AM	268h	72	Elevated Breathing	26	8	51	Check SpO2 and lung sounds; assess for fluid overload or early infection

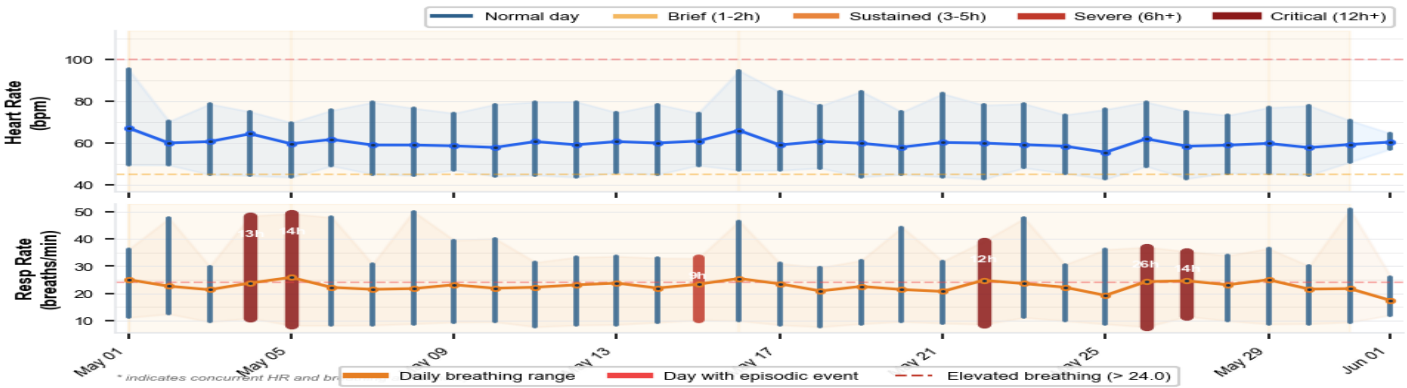
Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

Suggested Clinical Review Actions

- Elevated Breathing (avg 26 brpm, peak 51 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Breathing (avg 31 brpm, peak 34 brpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 5 unstable phases with 13 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.
- Daily recorded hours declined from 24.0 to 1.0 (96%) over the monitoring period.

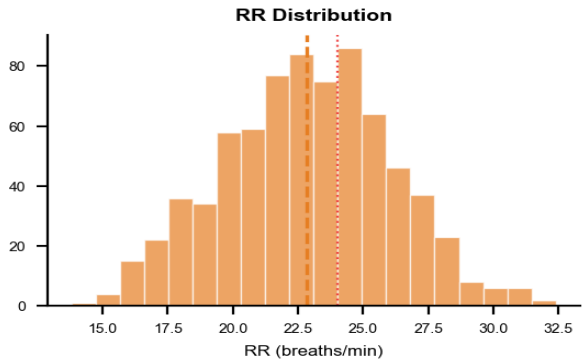
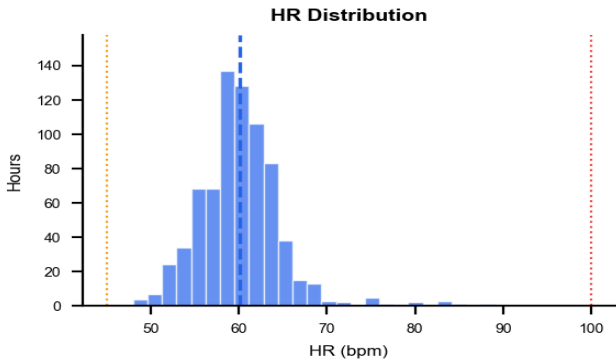
Clinical Pattern Observations:

- 1. Sustained finding: 14h continuous Elevated Breathing on May 05.
- 2. Breathing variability: RR spread 10 brpm (17 to 27).
- 3. Clustered pattern: Episodic events on 6 of 32 days (18%).

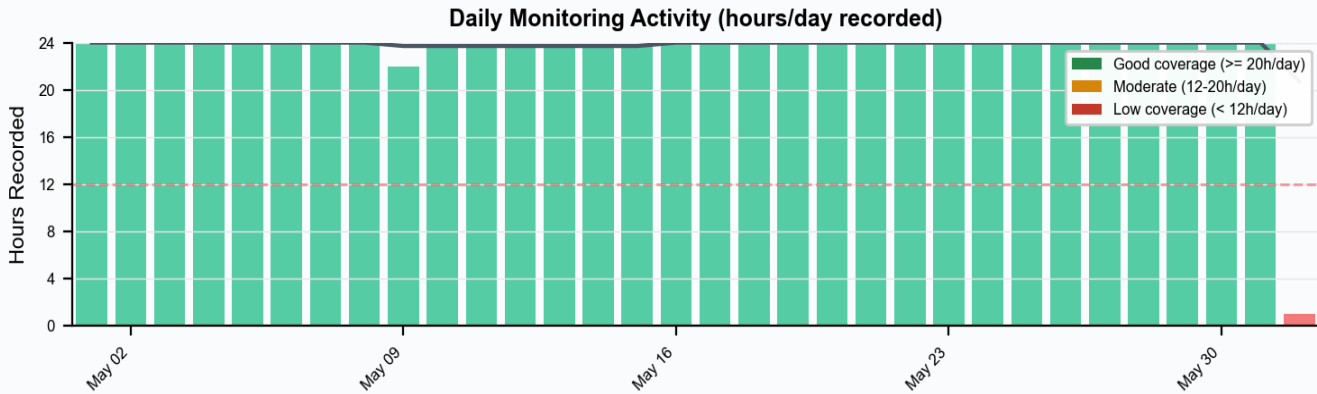


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

HR episode RR episode Recorded, no episode No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.